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(iii) Methods to be used

I have no real feelings regarding the selection and investigation methods which are described for patients, relatives and controls. It would not be ethical in most U.K. centres to offer financial inducements. This is referred to as a "stipend" in the consent document.

The applicant seeks to measure debrisoquine and 4-hydroxy-debrisoquine in up to 3000 urine specimens and has as yet not developed locally any expertise in this assay. I find this a little disturbing since they also plan to deviate from the established electron-capture assay which has been widely employed in the U.K. and utilise a flame-ionisation detection method. In addition, samples of authentic debrisoquine, 4-hydroxy-debrisoquine and the internal standard are not widely available and I am presently unaware of any approach to obtain them. It is possible that the applicant intends to carry out some chemical synthesis de novo. In any case, I find it flippant that the possibility of having a working assay, upon which the project would depend completely, has not yet been better explored by the applicant.

The statistical methodology appears adequate. However, better epidemiologic advice could be taken so that the maximum value could be obtained from such a study, for example lifestyle and dietary questionnaire and family history of disease.

(iv) Funding and resources sought

I found it hard to interpret some of the financial requests. If I am not mistaken, the applicant seeks funding for the salaries of 19 workers including himself, running costs and capital expenditure to the sum total of approximately \$3.2 million over three years. To put this into proper perspective, the study which Dr Calabrese seeks to confirm was carried out by a clinical PhD student who also did all the laboratory analysis. I would estimate that no more than \$70,000 were spent on it.

Conclusion

The application is both naive and repetitive, It does not serve to advance our knowledge of cancer cause. It makes no intellectual steps above and beyond our own published work on the subject. I can only conclude that in some way Dr Calabrese's unit is under threat and that this application is the best he can produce to underpin its funding.

Finally, you should know that the NIH have commissioned a study of debrisoquine hydroxylation in cancer and control patients which began in 1985 and will, amongst other things, answer all the questions posed in this application.

Yours sincerely



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